

Leadership Context Statement, Organizational Leadership, and Influence

George M. Hoganson, MD, EMBA60

Introduction

Leadership, to me, is the disciplined practice of aligning people and systems toward a shared purpose. My professional context as a pediatric emergency physician and medical director operating at the intersection of academic medicine and health-system operations offers both opportunity and tension. It demands clarity in uncertainty, influence without formal authority, and the ability to translate institutional complexity into care that is safe, equitable, and deeply human. The following statement situates my leadership within that environment—what is working well, where challenges persist, and how I am developing the capacity to lead with clarity, sustainability, and purpose.

Current Role and Key Responsibilities

I serve as the Medical Director for Washington University and St. Louis Children's Hospital Convenient Care Clinics, a network of three after-hours pediatric sites across Missouri and Illinois that collectively see more than 28,000 patient visits annually. These clinics provide an essential bridge between primary care and the emergency department, expanding access for families while optimizing hospital throughput and resource utilization.

In this role, I provide strategic and medical leadership, overseeing quality, safety, and operational performance while guiding staffing, documentation, and clinical guideline implementation. I partner closely with Washington University School of Medicine and BJC HealthCare leadership to align daily operations with broader institutional priorities. My scope also includes strategic planning and capacity analysis for potential expansion into a broader pediatric urgent-care service line.

I remain clinically active as a Pediatric Emergency Medicine attending at St. Louis Children's Hospital, a high-acuity Level I Pediatric Trauma Center serving a multi-state region. Remaining at the bedside sustains both credibility and perspective, and most importantly caring for people directly, at the bedside is my passion. This combination of operational leadership and clinical immersion defines my work—navigating the interface between the clinical, academic, and administrative cultures that together shape our mission.

What Is Working Well

The greatest alignment between my values and my role lies in improving systems of care for children and families. Under my leadership, the Convenient Care Clinics have experienced a 35 percent growth in patient volume while sustaining high satisfaction and safety outcomes. These results reflect not only sound operational design but an ethos of shared purpose, transparency, and team-based improvement.

I find particular energy in building interdisciplinary partnerships that cross organizational boundaries. Collaborations with nursing, quality-improvement, and administrative leaders have advanced triage accuracy, standardized documentation, and strengthened after-hours access. Together, these experiences have affirmed my belief that meaningful progress in healthcare emerges not only from technical fixes but from the trust, clarity, and communication that allow teams to translate purpose into performance.

Equally, I take pride in leading with presence and steadiness. In high-stakes or uncertain moments, I aim to bring calm and clarity so others can perform at their best. Over time, this approach has built

credibility, fostered openness, and created psychological safety—conditions that enable continuous improvement and shared accountability.

Current Challenges

My work is shaped by the complexity of operating within dual academic and hospital systems. Washington University and BJC HealthCare each possess distinct hierarchies, decision rhythms, and success metrics. While this duality generates innovation, it also demands constant translation and diplomacy to maintain alignment across institutional lines.

Another challenge involves balancing clinical urgency with organizational consensus. Emergency medicine requires decisive, hierarchical action, whereas institutional governance emphasizes deliberation and inclusivity. Learning when to lead through direction and when through facilitation has become a continual exercise in adaptive leadership.

Finally, as my sphere of influence expands, I am learning to scale leadership through others. Sustaining results now depends on culture, communication, and systems more than on personal oversight. I have come to view these challenges as a crucible for growth—pushing me to shift from solving problems to enabling others to solve them, from doing to designing, from clarity for the moment to clarity that endures.

These lessons have clarified not only how I lead today, but the kind of leader I hope to become—one whose influence scales through systems, culture, and the development of others.

Professional Goals (3–5 Years)

Over the next three to five years, I aspire to advance into a broader system-level leadership role—for example, as Chief Medical Officer or comparable institutional leader—focused on improving access, quality, and workforce sustainability across the continuum of pediatric care. These ambitions are not about positional advancement alone but about influence—shaping how care systems function, how people experience work, and how culture sustains purpose under pressure.

My long-term goal is to translate frontline insight into scalable, evidence-based system design, aligning care delivery with equity and operational efficiency. I measure success not only by scope of responsibility but by the strength of the systems and people around me—improvements in throughput and access balanced with greater engagement, trust, and psychological safety among teams.

My Executive MBA experience has refined this vision, equipping me with the analytical and strategic tools to link values-based leadership with measurable outcomes. I intend to apply those insights to bridge the cultures of medicine, management, and public health—creating systems that are both effective and humane. Sustaining this trajectory will require ongoing reflection and learning—remaining curious, adaptable, and humble enough to keep evolving alongside the system I serve.

Strengths

My leadership is grounded in three enduring strengths:

1. **Purpose-Driven Leadership Grounded in Values.** I lead through service, integrity, and compassion, guided by the conviction that improving care for children and supporting those who deliver it is both my work and my purpose.
2. **Systems Thinking and Interdisciplinary Collaboration.** I see patterns across domains and thrive in complexity. This systems mindset enables me to connect operational and human factors, turning institutional friction into coordinated progress.

3. **Emotional Presence and Humanism.** I value steadiness, empathy, and trust. Psychological safety and relational integrity are, to me, not soft skills but the foundation of sustainable performance.

These strengths form the foundation of my leadership identity and shape how I navigate high-stakes environments with steadiness and empathy while aligning people around shared outcomes.

Developmental Edges

My developmental needs are, in many ways, the inverse of my strengths—reminders that every quality, when overextended, can become a constraint.

1. **Visibility and Strategic Self-Advocacy.** My instinct for humility and collective success can obscure my contributions. I am learning that visible leadership—anchored in purpose rather than ego—helps align others and secure support for institutional goals. I am cultivating this skill through structured updates, purposeful communication, and mentorship from senior leaders.
2. **Boundaries That Sustain Compassion.** Empathy has long been a strength, but without boundaries it can lead to depletion. I am developing habits of reflection, pacing, and rest that preserve presence without overextension—recognizing that sustainable compassion strengthens teams.
3. **Focus Amid Multipotentiality.** My curiosity spans disciplines and opportunities. While this breadth fuels creativity, I am practicing discernment—using quarterly reflection and values-based filters to ensure energy flows toward the work of highest institutional and personal impact.

These are as much capacities to refine as deficiencies to improve upon—integrating intentional visibility, sustainable empathy, and strategic focus to expand both reach and resilience.

Core Values

The values that guide my leadership are Purpose, Integrity, and Compassion.

- Purpose centers every decision on improving systems that serve children and families—helping people and the systems that deliver care reach their full potential.
- Integrity ensures consistency between what I say and what I do, building trust in moments of complexity.
- Compassion defines how I lead—meeting people with empathy, listening deeply, and honoring their contributions.

These values are not theoretical—they are practiced daily in the decisions, tradeoffs, and conversations that define leadership. They remind me that outcomes matter most when achieved in ways that dignify people and strengthen the whole.

Conclusion

My leadership context is defined by complexity and connection but also by opportunity.

Operating within the overlapping cultures of clinical care, academia, and administration has taught me that success depends less on uniformity than on harmonizing difference—translating between precision, deliberation, and efficiency so that each domain strengthens the others.

I am most engaged where medicine meets management, where data meets story, and where human connection transforms systems. My aspiration is to lead with clarity, compassion, and curiosity—to build teams and systems that not only perform but endure, and to deliver care that honors both the science and the humanity that define our mission.

Table 1. Span of Leadership and Mentorship

Category	Count	Description
Direct Reports	4	1 Operations Manager, 2 Physicians, 1 PEMRAP Program Coordinator
Indirect Reports	≈70	3 Nurse Managers, 25 APNs, 32 RNs, 10 MAs (Convenient Care)
Educational / Research Influence	≈118	100 PEMRAP Research Associates and 18 Non-PEMRAP Mentees (Undergraduate–Fellow)
Total Individuals Under Leadership Influence	≈192	Across clinical, operational, and academic missions at WashU and BJC

Table 1. Summary of leadership span across operational and academic domains, including direct supervision, indirect oversight, and educational mentorship.

Washington University School of Medicine (Academic Line)

BJC HealthCare / St. Louis Children's Hospital (Operations Line)

